

Name	: Ms. PUVIARASI	Age	: 44 Years
Lab No.	: 445629943	Gender	: Female
Ref By	: SELF	Reported	: 2/5/2024 2:24:21PM
Collected	: 1/5/2024 7:24:00AM	Report Status	: Final
A/c Status	: P	Processed at	: LPL-CHENNAI
Collected at	: CHENNAI LAB HOME VISIT		
	KKR ENT Hospital & Research Institute		KKR ENT Hospital & Research Institute
	building,2nd Floor,#6OMR Road Karapakkam		Building, 2nd floor, 6 OMR road,
	CHENNAI 600096		Karapakkam, chennai - 6000096



Test Report

Test Name	Results	Units	Bio. Ref. Interval
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SWASTHFIT COMPLETE PACKAGE

LIVER & KIDNEY PANEL, SERUM

Creatinine (Modified Jaffe,Kinetic)	0.64	mg/dL	0.51 - 0.95
GFR Estimated (CKD EPI Equation 2021)	111	mL/min/1.73m2	>59
GFR Category (KDIGO Guideline 2012)	G1		
Urea (UREASE UV)	18.81	mg/dL	14.9 - 38.5
Urea Nitrogen Blood (UREASE UV)	8.78	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio (Calculated)	14		
Uric Acid (Uricase)	3.31	mg/dL	2.60 - 6.00
AST (SGOT) (IFCC)	24.0	U/L	<35
ALT (SGPT) (IFCC)	13.5	U/L	<35
GGTP (GCNA)	17.6	U/L	<38
Alkaline Phosphatase (ALP) (IFCC AMP-BUFFER)	63.03	U/L	30 - 120
Bilirubin Total (DPD)	0.31	mg/dL	<1.00
Bilirubin Direct (DPD)	0.08	mg/dL	0.00 - 0.30
Bilirubin Indirect (Calculated)	0.23	mg/dL	<1.10
Total Protein (Biuret)	6.92	g/dL	6.40 - 8.30
Albumin (BCG)	3.92	g/dL	3.50 - 5.20
A : G Ratio (Calculated)	1.31		0.90 - 2.00
Globulin(Calculated) (Calculated)	3.00	gm/dL	2.0 - 3.5
Calcium, Total (Arsenazo III)	9.18	mg/dL	8.60 - 10.30



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Test Name	Results	Units	Bio. Ref. Interval
Phosphorus (Molybdate UV)	3.60	mg/dL	2.40 - 4.40
Sodium (Indirect ISE)	137.20	mEq/L	136.00 - 146.00
Potassium (Indirect ISE)	4.40	mEq/L	3.50 - 5.10
Chloride (Indirect ISE)	100.60	mEq/L	101.00 - 109.00



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Test Name	Results	Units	Bio. Ref. Interval
LIPID SCREEN, SERUM			
Cholesterol, Total (CHO-POD)	225.73	mg/dL	<200.00
Triglycerides (GPO-POD)	63.43	mg/dL	<150.00
HDL Cholesterol (CHO-POD)	38.87	mg/dL	>50.00
LDL Cholesterol, Calculated (Calculated)	174.17	mg/dL	<100.00
VLDL Cholesterol, Calculated (Calculated)	12.69	mg/dL	<30.00
Non-HDL Cholesterol (Calculated)	187	mg/dL	<130

Note

- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL& LDL Cholesterol.
- Additional testing for Apolipoprotein B, hsCRP, Lp(a) & LP-PLA2 should be considered among patients with moderate risk for ASCVD for risk refinement.

Treatment Goals as per Lipid Association of India 2020

RISK CATEGORY	TREATMENT GOAL		CONSIDER THERAPY	
	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)
Extreme Risk Group Category A	<50 (Optional goal ≤30)	<80 (Optional goal ≤60)	≥50	≥80
Extreme Risk Group Category B	≤30	≤60	>30	>60
Very High	<50	<80	≥50	≥80
High	<70	<100	≥70	≥100
Moderate	<100	<130	≥100	≥130
Low	<100	<130	≥130*	≥160*

*In low risk patient, consider therapy after an initial non-pharmacological intervention for at least 3 months



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE, FASTING (F) (Hexokinase)			
Glucose Fasting	92.50	mg/dL	70.00 - 100.00
VITAMIN B12; CYANOCOBALAMIN (CLIA)			
Vitamin B12; Cyanocobalamin	148.00	pg/mL	180.00 - 914.00

Interpretation

Remarks	Result In pg/mL
Normal	180 - 914
Indeterminate	120 - 180
Deficient	< 120

Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity

VITAMIN D, 25 - HYDROXY, SERUM (CLIA)			
Vitamin D, 25 Hydroxy	35.53	nmol/L	75.00 - 250

Interpretation



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LEVEL	COMMENTS		
Deficient	High risk for developing bone disease		
Insufficient	Vitamin D concentration which normalizes Parathyroid hormone concentration		
Sufficient	Optimal concentration for maximal health benefit		
Potential intoxication	High risk for toxic effects		

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available

THYROID PROFILE,TOTAL, SERUM (CLIA)

T3, Total	1.31	ng/mL	0.70 - 2.04
T4, Total	9.58	µg/dL	5.74 - 13.03
TSH	2.39	µIU/mL	0.34 - 5.60

Note

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
- Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
- Unbound fraction (Free,T4 /Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration
- Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals



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Test Name	Results	Units	Bio. Ref. Interval
IRON STUDIES, SERUM (Spectrophotometry, TPTZ, NITROSO - PSAP)			
Iron	43.00	µg/dL	50.00 - 170.00
Total Iron Binding Capacity (TIBC)	395.00	µg/dL	250.00 - 450.00
Transferrin Saturation	10.89	%	15.00 - 50.00



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD (HPLC, NGSP certified)			
HbA1c	6.1	%	4.00 - 5.60
Estimated average glucose (eAG)	128	mg/dL	

Interpretation

HbA1c result is suggestive of at risk for Diabetes (Prediabetes)/ well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults >=18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	>= 6.5	<7.0

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants,elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g.,recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used.Iron deficiency anemia is associated with higher HbA1c



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Test Name	Results	Units	Bio. Ref. Interval
Physical			
Chemical			
URINE EXAMINATION, ROUTINE; URINE, R/E	Sample Not Received		
Microscopy			



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Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM (Photometry,Electrical Impedence,VCS Technology ,Calculated & westergren)			
Hemoglobin	10.70	g/dL	12.00 - 15.00
Packed Cell Volume (PCV)	34.30	%	36.00 - 46.00
RBC Count	5.01	mill/mm3	3.80 - 4.80
MCV	68.40	fL	83.00 - 101.00
MCH	21.40	pg	27.00 - 32.00
MCHC	31.20	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW)	19.80	%	11.60 - 14.00
Total Leukocyte Count (TLC)	7.35	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	60.81	%	40.00 - 80.00
Lymphocytes	29.47	%	20.00 - 40.00
Monocytes	7.31	%	2.00 - 10.00
Eosinophils	2.15	%	1.00 - 6.00
Basophils	0.26	%	<2.00
Absolute Leucocyte Count			
Neutrophils	4.47	thou/mm3	2.00 - 7.00
Lymphocytes	2.17	thou/mm3	1.00 - 3.00
Monocytes	0.54	thou/mm3	0.20 - 1.00
Eosinophils	0.16	thou/mm3	0.02 - 0.50
Basophils	0.02	thou/mm3	0.02 - 0.10
Platelet Count	309	thou/mm3	150.00 - 410.00
Mean Platelet Volume	7.8	fL	6.5 - 12.0
E.S.R.	5	mm/hr	0.00 - 20.00



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Test Report

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Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood

Dr.Jackson Bose
M.D. Pathology
Consultant Pathologist
Dr Lal PathLabs Ltd



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,BANGALORE-560103



Test Report

Test Name	Results	Units	Bio. Ref. Interval
SWASTHFIT COMPLETE PACKAGE			
C-REACTIVE PROTEIN, CARDIO; hsCRP (Immunoturbidimetry)	5.20	mg/L	<1.00

Interpretation

CARDIO CRP IN mg/L	CARDIOVASCULAR RISK
<1	Low
1-3	Average
3-10	High
>10	Persistent elevation may represent Non cardiovascular inflammation

APOLIPOPROTEINS A1 & B, SERUM (Immunoturbidimetry)			
Apolipoprotein (Apo A1)	109	mg/dL	105.00 - 205.00
Apolipoprotein (Apo B)	115	mg/dL	55.00 - 130.00
Apo B / Apo A1 Ratio	1.06		0.35 - 0.98

As per recommendations of National Cholesterol Education Program (NCEP) the clinical significance of results is as follows:

Apolipoprotein B

RESULT IN mg/dL	REMARKS
<23	Abetalipoproteinemia/Hypobetalipoproteinemia
23-45	Hypobetalipoproteinemia
46-135	Normal
>135	Hyperapobetalipoproteinemia/Increased CAD risk

Apo B to A1 Ratio

RATIO	REMARKS
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0.35-0.98	Desirable		
>0.98	Increased CAD risk		



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AMYLASE, SERUM (IFCC)			
Amylase	100.00	U/L	28.00 - 100.00

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-----End of report -----



IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory.
•Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.
(#) Sample drawn from outside source.
If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.
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